[redacted]

FAX

DA[redacted] Time: _____

TO: Illumination Foundation

Fax Number: [redacted]

From: [redacted]

Page(s): 39 including fax coversheet

R[redacted]

☐ Urgent ☒ For Review ☐ Pls. Comment ☐ Pls. Reply ☐ Pls. Recycle

Note/Comments:

Please email me [redacted]

CONFIDENTIAL NOTICE:

THE DOCUMENT ACCOMPANYING THIS FACSIMILE TRANSMISSION [redacted] CONTAIN CONFIDENTIAL, PRIVELEGED INFORMATION, BELONING TO THE SENDER. THE ORIGINAL RECORDS WILL BE MAINTAINED IN THE FACILITY. IF YOU ARE NOT THE INTENDED RECIPIENT, YOU HEREBY NOTIFIED THAT ANY DISCLOSURE, COPYING, DISTRIBUTION OR THE TAKING OF ANY ACTION IN RELIANCE ON THE CONTENTS OF THIS FAXED INFORMATION IS STRICTLY PROHIBITED. IF YOU HAVE RECEIVED THIS FAX IN ERROR, PLEASE IMMEDIATELY CONTACT BY TELEPHON[redacted] ANGE FOR THE RETURN OF THE DOCUMENT.

ILLUMINATION
RECUPERATIVE

LA/OC REFERRAL FORM

To refer a patient, please complete the entire form including supporting documents and FAX to (888)382-9551 or email to RECUP@ilhomeless.org For all referral questions, please contact IF Referral Coordinator at (888)505-0855

Referring Hospital: [redacted] Dept/Floor: _____
SW/CM/PN: [redacted] Phone/pager#: _____
E-mail: [redacted] Nursing st phone#: [redacted]
Authorized by: _____

Patient Name: [redacted] MRN _____ Date of Birth: ____/____/____

Gender: ☐ Male ☐ Female ☐ Transgender Ethnicity: _____ Social Security# (last 4): xxx-xx-____

☐ ED Visit ☒ Hospital Admit [redacted] Expected Discharge Date [redacted]

Insurance Type: _____ English Speaking? ☒ Yes ☐ No *Primary Language Spoken: Eng

PLEASE VERIFY: ☒ Currently homeless Check if: ☐ Homeless for more than 1 year continuously or 4 episodes in last 3 years

CHIEF COMPLAINT/ADMITTING DIAGNOSIS: Major Depressive Disorder

SUBSTANCE ABUSE? ☒ None ☐ Alcohol ☐ Cocaine ☐ Heroin ☐ Meth ☐ Other: _____
Last Date Used: ____/____/____ Current withdrawals: ☐ No ☐ Yes *Please explain _____

ANY WOUNDS? ☐ Yes ☒ No #/Loc/Size/Stage: _____

Independent with wound care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ no

PPD/TB test or Chest XRAY performed? ☐ yes ☐ no Date: _____ Results: _____

ANY LIMITATIONS, BEHAVIORAL CHALLENGES OR MENTAL HEALTH ISSUES? ☐ None ☐ Auditory/Visual Hallucinations

☒ Mental Health DX IF yes, what type?: Major depressive disorder ☐ Non-compliant ☐ forgetful

☐ Cognitive Impairment ☐ Reg. Sex offender ☐ other, please explain _____

Requires O2? ☐ Yes ☒ No Self Administer All Meds? ☒ Yes ☐ No *Please explain: _____

Continent of Bowel & Bladder? ☐ Yes ☒ No Colostomy/Ileostomy ☐ Yes ☒ No Foley Catheter ☐ Yes ☒ No

Independent w/all ADL's? ☒ Yes ☐ No *Please explain: _____

Diabetic? ☒ No If ☐ Yes, then: ☐ Insulin ☐ Oral Meds Communicable Diseases?: _____

Anticoagulants?: ☒ No IF ☐ yes, Requires INR/PT/PTT checks through Home Health or Clinic? _____

Ambulatory? ☒ Yes ☐ No Assistive device? ☒ No ☐ Yes *Please Explain: _____

Does Patient have a car? ☐ Yes ☒ No Spouse/Partner? ☐ Yes ☒ No Service Animal? ☐ Yes ☒ No

Length of Stay authorized in Recuperative Care: ____ days

Attached Docs: ☐ Face Sheet ☐ H&P ☐ Med List ☐ Psych notes ☐ Surgical notes ☐ PT/OT Eval. ☐ SW notes

IF STAFF ONLY

Date Received: _____ Reviewed by: _____ Approved: ☐ Yes ☐ No Date: _____ Auth# _____

Denied Reason?: _____ Admission Date/time: _____ Exit Date: _____

Extension Requested?: ☐ no ☐ yes If yes, Approved additional days?: _____ by: _____ TOTAL LOS: _____

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CalOptima Health

CalAIM Recuperative Care/Short-Term Post Hospitalization Referral/Authorization Request

Instructions:
Please complete this form and attach the following documents: Plan of Care, Discharge Plan, PT/OT notes and H&P. Authorization [redacted] be denied if this information is not provided.

Step 1: Complete all information below.

Referral Information

Referral date: [redacted] Referred by: [redacted]

Agency or relationship to membe[redacted]

Referring provider National Provider Identifier (NPI) (if applicable):

Phone [redacted] Fax: Email: [redacted]

Member Infor[redacted]

Member name [redacted] [redacted] Health ID: [redacted]

Date of birth: [redacted] Member's preferred language: English

Phone: Email:

Is the member currently in hospital? ☒ Yes ☐ No

Last visit to PCP: Next PCP visit:

Primary Care Provider (PCP) Information

Name: [redacted]

Phone: Email:

Step 2: [redacted] the boxes for the Community Supports the member is interested in receiving. The following pages provide additional eligibility information about Community Supports.

Step 3: Please check off the most appropriate Recuperative Care/Short-Term Post-Hospitalization Housing pathways based on the eligibility listed in the following pages.

Please note that members [redacted] not receive more than a combined 182 days of Short-Term Post-Hospitalization, Recuperative Care and Transitional Rent during any rolling 12-month period.

Pathway	Description	Eligibility
☑ Recuperative Care only	Short-term residential care for individuals who no longer require hospitalization but will need to heal from an injury, illness or mental health condition.	Both must apply: Homeless or at risk of homelessness Member requires recovery in order to heal from an injury or illness.
☐ Short-Term Post-Hospitalization Housing only	Assists members with high medical or behavioral health needs with short-term housing after leaving the hospital, recovery facility, recuperative care or other facility.	Member must meet the following criteria: (1) Member is exiting an institution, which includes recuperative care facilities, inpatient hospitals, residential substance use disorder or mental health treatment facilities, correctional facilities or nursing facilities. **AND** (2) Member is experiencing homelessness. **AND** (3) Member meets one the following criteria: a. Is receiving ECM b. Have one or more serious chronic conditions c. Have a serious mental illness; or d. Is at risk of institutionalization or requiring residential services as a result of a substance use disorder. **AND** (4) Member is having ongoing physical and behavioral health needs as determined by a qualified health professional that would otherwise require continued institutional care if not for receipt of Short-Term Post-Hospitalization Housing.
☐ Nursing Facility with plans to transition to Recuperative Care	Short-term residential care for individuals who no longer require hospitalization but still need to heal from an injury or illness or a mental health condition.	Select one that applies: Homeless or at risk of homelessness Member who is at risk of hospitalization or at post-hospitalization Members who live alone with no formal supports.

☐	Recuperative Care with plans to transition to Short-Term Post-Hospitalization Housing	Short-term residential care for individuals who no longer require hospitalization but still need to heal from an injury, illness or mental health condition.	Select one that applies: Homeless or at risk of homelessness Individuals who are at risk of hospitalization or at post-hospitalization **OR** Individuals who live alone with no formal supports **AND** Member is exiting recuperative care, inpatient hospital, residential substance use disorder treatment facility, residential mental health treatment facility, correctional facility or nursing facility.

Admitting Diagnosis

Qualifying Recuperative Care/Short-Term Post-Hospitalization Housing diagnosis:

ED visit/hospital admit date: Expected discharge date:

Will the member need any specialist follow-up care? ☐ Yes ☑ No

1) Specialty:
   Provider name:
   Phone:
   Scheduled appt date:
2) Specialty:
   Provider name:
   Phone:
   Scheduled appt date:
3) Specialty:
   Provider name:
   Phone:
   Scheduled appt date:

Authorized Home Health Provider

Service(s): ☐ Physical therapy ☐ Occupational therapy ☐ Speech ☐ Wound care

Health Information

Does the member have a preferred pharmacy? ☐Yes ☑No
If yes, where:
Does the member understand how to take their medication and why they are taking their medication? ☑Yes ☐No

Step 4: Based on the services selected for the member above, please submit this Referral Form to the most appropriate provider listed below via fax or mail.

Community Supports Provider Contact Information
Recuperative Care and Short-Term Hospitalization

[redacted]

[redacted]mination Foundation (serving adults and children)
Phone number: [redacted]
Fax number: [redacted]
Email: recup@ifhomeless.org

Select from the below only after checking the capacity for OC Providers first.

[redacted]

STOP

Community Supports Provider to Complete Section Below

Step 5: Complete the section below and return the response to the referrer at the hospital or skilled nursing facility. If the member belongs to Kaiser Permanente, please submit these documents directly to Kaiser Permanente.

Accepting/Not Accepting

Was the member accepted? ☐Yes ☐No
If the member [redacted] service, please provide the reason:

Infections

Communicable disease/isolation describe:

IV Antibiotics: ☐ Yes ☑ No
If yes, frequency:

Psycho-social Information:

☐ Registered sex offender

Member has:
☐ Car ☐ Spouse/partner
☐ Service animal ☐ Pets

Substance Use ☑ None

Type	Last Date Used
☐ Alcohol	
☐ Cocaine	
☐ Heroin	
☐ Methamphetamine	
☐ Opioid	
☐ Other	

Mental Health DX

☐ Anxiety ☐ Bipolar ☐ Cognitive impairment ☐ Depression ☐ Schizophrenia
☐ Trauma-related ☐ Other (please explain):

☐ Current treatment:

Requires assistance with ADLs, please explain: NO

Medication Management

☐ Diabetic ☐ Insulin ☑ Oral meds ☐ Anticoagulants ☐ Requires INR/PTT checks
☐ Requires assistance with medication. List medication(s):

Does the member have enough medication to last through the end of the month? ☑ Yes ☐ No
Does the member understand how to obtain refills on their medications? ☑ Yes ☐ No

☐ Personal care
Provider name: [redacted] [redacted] appt date:

Health Information

General

Gender: ☐ Male ☑ Female
☐ Placement needs related to gender.

COVID vaccine
Dose 1: ☐ Yes ☐ No
Dose 2: ☐ Yes ☐ No
Booster 1: ☐ Yes ☐ No
Booster 2: ☐ Yes ☐ No

TB test or chest X-ray performed? ☐ Yes ☐ No
If Yes, date:
Results: ☐ Positive
☐ Negative
Comments:

Neuro

Alert and oriented to: ☐ Person ☐ Place ☐ Time ☐ Situation

Respiratory

☐ Requires O2 (Explain):

GI/GU

☐ Incontinent of bowel
☐ Incontinent of bladder
☐ Colostomy/ileostomy
☐ Foley catheter

Does the member require tube feeding?
☐ Yes ☑ No

Ambulation/Mobility

Can the member independently perform ADLs? ☑ Yes ☐ No
How far can the member ambulate?

Does the member use DME? ☐ Yes ☑ No
If yes, please explain:
Fractures: ☐ Yes ☑ No

Recent surgery: ☐ Yes ☑ No

Integumentary

Wound(s): ☐ Yes ☑ No
If yes, provide location(s)/size/stage:

Independent with wound care? ☐ Yes ☐ No

Face Sheet

[redacted]

RESIDENT INFORMATION

[redacted]							
[redacted]		[redacted]		[redacted]		United States	
				Race		Occupation	Primary Lang.
F	[redacted]	74	Single	None	White		English
				Religion			Maiden Name
Acute care hospital		[redacted]					
[redacted]						Medicare Name	
	[redacted]					Managed Care Policy #	
Managed Medicaid #		Medicaid # / Social Security		Ins. Carrier		Effective Date	

PAYER INFORMATION

Primary Payer	Medicare A	Policy #		Group	null	Ins. Company	Medicare
Secondary Payer	Managed Medi-Cal QMB Crossover A	Medicare #		Copay		Ins. Company	CAL-OPTIMA

OTHER INFORMATION

	Allergies		
[redacted]	Penicillin		
Admission Type	Advance Directive	Attending Physician	Emergency Contact
Ed Visit	Hospice	Laundry Code #	Medi-Cal Beneficiary
Medicare Terminal Date	Medicare Secondary Coverage	Medical Worker	Medicare Coverage
Last Medical Information	Patient Comfort	Preferred Pharmacy	Legal Guardian/Emergency Contact
QMB Eligible	Spouse's Name	Social Worker	Uses Wheelchair?
Veteran			

CARE PROVIDERS

[redacted]

PHARMACY

Primary Pharmacy		Address	

PHARMACY

[redacted]

EXTERNAL FACILITIES (No Data Found)

Facility Name	Phone	Facility Type

CONTACTS

Name	Contact Type	Relationship	Phone
[redacted]	Responsible For Billing Statement Resident	Self	[redacted]
	Emergency Contact # 1	Son	

DIAGNOSIS INFORMATION

Code	Description	Code Set	Type	Classification
M51.35	OTHER INTERVERTEBRAL DISC DEGENERATION, THORACOLUMBAR REGION	[redacted]	Primary (#67)	Admitting Diagnosis (#69)
F32.9	MAJOR DEPRESSIVE DISORDER, SINGLE EPISODE, UNSPECIFIED		Principal	Admission
R26.81	UNSTEADINESS ON FEET		Diagnosis 3	Admission
M62.81	MUSCLE WEAKNESS (GENERALIZED)		Diagnosis 4	Admission
E46	UNSPECIFIED PROTEIN-CALORIE MALNUTRITION		Diagnosis 5	Admission
F03.90	UNSPECIFIED DEMENTIA, UNSPECIFIED SEVERITY, WITHOUT BEHAVIORAL DISTURBANCE, PSYCHOTIC DISTURBANCE, MOOD DISTURBANCE, AND ANXIETY		Diagnosis 6	Admission
[redacted]	ATHEROSCLEROTIC HEART DISEASE OF NATIVE CORONARY ARTERY WITHOUT ANGINA PECTORIS		Diagnosis 7	Admission
G47.00	INSOMNIA, UNSPECIFIED		Diagnosis 8	Admission
F41.9	ANXIETY DISORDER, UNSPECIFIED		Diagnosis 9	Admission
F29	UNSPECIFIED PSYCHOSIS NOT DUE TO A SUBSTANCE OR KNOWN PHYSIOLOGICAL CONDITION		Other Diagnosis	Admission
Z59.00	HOMELESSNESS UNSPECIFIED		Other Diagnosis	Admission

ADVANCE DIRECTIVE

POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

MISCELLANEOUS INFORMATION

Date of Change	Time	Length of Stay	Comment (Additional Information)
		36	

Signature	Date	Time

Patient Effects Service	Relationship	Date	Time

[redacted] Pharmacy [redacted] Physician [redacted]

REGULAR diet 7-Regular texture, 0 - Thin Liquid consistency

dical Conditions

MAJOR DEPRESSIVE DISORDER, SINGLE EPISODE, UNSPECIFIED(F32.9), UNSTEADINESS ON FEET(R26.81), MUSCLE WEAKNESS (GENERALIZED)(M62.81), UNSPECIFIED DEMENTIA, UNSPECIFIED SEVERITY, WITHOUT BEHAVIO DISTURBANCE, PSYCHOTIC DISTURBANCE, MOOD DISTURBANCE, AND ANXIET[redacted]ATHEROSCI[redacted]HEART DISEASE OF NATIVE CORONARY ARTERY WITHOUT ANGINA PECTORIS(I25.10), INSOMNIA, UNSPECIFIED D(G 00), ANXIETY DISORDER, UNSPECIFIED(F41.9), HOMELESSNESS UNSPECIFIED([redacted]), UNSPECIFIED [redacted]SIS NOT DUE TO A SUBSTANCE OR KNOWN PHYSIOLOGICAL CONDITION(F29), UNSPECIFIED PROTEIN-CALOR E MALNUTRITION(E46), OTHER INTERVERTEBRAL DISC DEGENERATION, THORACOLUMBAR REGION(M51.35)

vance Directive

POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]6

Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon
1																													

apro[redacted]
e 5 mg by mouth at bedtime for
ression m/b verbalization of sadness
ified informed consent obtained by
.
der Date [redacted]

2100 [redacted]

itor for Orthostatic Hypotension daily
lays then weekly on Sundays during
Shift. Monitor B/P while LYING
A/N.
ry day shift every Sun - Notify MD of
mm/Hg difference in SBP and
mm/Hg for Diastolic B/P
[redacted]

BP	X	X	X	X	X	X	126/	X	X	X	X	X	X	124/															
Day	X	[redacted]																											

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
-- Follow Up Codes -- √=Administered I=Ineffective E=Effective U=Unknown H=On Hold By Physician ---- Chart Codes ---- (Refer to the last page of the report for a complete list of chart codes) GA=Group Observed - All OI=Observed Individual	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameter Administration 5=Hold/See Progress Notes							Checked By 1st		
								Checked By 2nd		
								[redacted]cked By 3rd		

MEDICATI[redacted]

Admit Date [redacted]

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for 26	Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
itor for Orthostatic Hypotension daily lays then weekly on Sundays during Shift. Monitor B/P while SITTING. ry day shift every Sun - Notify MD of m/Hg difference in SBP and m/Hg for Diastolic B/P [redacted]	BP	X	X	X	X	X	X	125/74	X	X	X	X	X	X	125/73	X	X	X	X	X	X	X	X	X	X	X	X	X		X
	*Day	X	[redacted]																											
itor for Orthostatic Hypotension daily lays then weekly on Sundays during Shift. Monitor B/P while STANDING. ry day shift every Sun - Notify m/Hg difference in SBP and m/Hg for Diastolic B/P [redacted]	BP	X	X	X	X	X	X	127/76	X	X	X	X	X	X	123/68	X	X	X	X	X	X	X	X	X	X	X	X	X		X
	*Day	X	[redacted]																											
neron Tablet 7.5 mg by mouth at bedtime for ression m/b Poor PO intake. Verified rmed consent obtained by MD. [redacted]	2[redacted]	[redacted]																												

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
— Follow Up Codes — V=Administered I=Ineffective E=Effective U=Unknown H=On Hold By Physician	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes							Checked By 1st		
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— Chart Codes — efer to the last page of the report a complete list of chart codes) GA=Group Observed - All GI=Observed Individual		MEDICAT[redacted] Admit Date								

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

	Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon
		1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29
:P 2: TB SKIN TEST (MANTOUX) 5U CC) INTRADERMAL x1 ry evening shift for 1 Day PPD skin must be read within 48 to 72 hrs. ument result in mm.	MM	X	X	X	X	X	X	X	X	X	X	X	X	X	Comm	X	X	X	X	X	X	X	X	X	X	X	X	X	X	Σ
	*Eve	X	[redacted]																											
[redacted]																														
:P 2: TB SKIN TEST (MANTOUX) 5U CC) INTRADERMAL x1 ument Lot# & Exp. date in progress : der Date:	Site	X	X	X	X	X	X	X	X	X	X	X	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	>
	Eve	X	[redacted]																											
[redacted]																														
SKIN TEST (MANTOUX) 5U (0.1CC) RADERMAL x1 ry evening shift for 1 Day PPD skin must be read within 48 to 72 hrs. ument result in mm.	MM	X	X	X	X	X	X	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	>
	*Eve	X	[redacted]																											
[redacted]																														
SKIN TEST (MANTOUX) 5U (0.1CC) [redacted] ument Lot# & Exp. date in progress	Site	X	X	X	X	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Eve	X	[redacted]																											
[redacted]																														

[redacted]

Chart Codes / Follow Up Codes

— Follow Up Codes —
√=Administered
I=Ineffective
E=Effective
U=Unknown
H=On Hold By Physician

— Chart Codes —
efer to the last page of the report a complete list of chart codes)
A=Group Observed - At
V=Observed Individual

OBP=Group Observed - Partial
1=Absent from facility without meds
2=Drug Refused
3=Absent from facility with meds
4=Vitals Outside of Parameters
Administration
6=Hold/See Progress Notes

Init	Name	Signature	Init	Name	Signature

[redacted]

MEDICATI[redacted]

Admit Date [redacted]

	Name	Signature
Checked By 1st		
Checked By 2nd		
Checked By 3rd	[redacted]	

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
SKIN TEST (MANTOUX) 5U (0.1CC) RADERMAL x1. REPEAT IN 7 DAYS IEGATIVE ry evening shift every 10 day(s) for 2 inistrations Document Lot# & Exp. [redacted]	Site	X	X	X	X	X	X	X	X	NA	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Eve	X	[redacted]																										
SKIN TEST (MANTOUX) 5U (0.1CC) RADERMAL x1. REPEAT IN 7 DAYS IEGATIVE ry evening shift every 10 day(s) for 2 inistrations PPD skin test must be l within 48 to 72 hrs. Document result [redacted]	MM	NA	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Eve	[redacted]																											

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
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– Chart Codes – efer to the last page of the report a complete list of chart codes) GA=Group Observed - All GI=Observed Individual		MEDICATI								
		Admit Date								

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted]

Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon
	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29
HRS	0	0	1	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
NPI	1234	1	1	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
OC	2	2	2	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
*Eve	[redacted]																												
HRS	8	0	7	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
NPI	1,2,3	123	123	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
OC	1	1	2	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
*Nigh	[redacted]																												
0900	[redacted]																												
1700	[redacted]																												

[redacted]

havior Monitoring Sedative/ Hypnotic
nazepam) - DX Insomnia m/b inability
leep at night. Monitor # of hours of
p. Interventions 1- Relaxation, 2-
ust room temp/ lighting, 3- Reposition,
oileting, 5- Music/TV, 6- Snack(s)/
comes: 1- Improved, 2- unchanged, 3-
sened.
ry evening and night shift for 14 Days

[redacted]

erDAL Tablet
e 1 mg by mouth two times a day for
chosis r/t medical condition AEB
den mood changes Verified informed
sent obtained by MD.

[redacted]

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
— Follow Up Codes — =Administered =Ineffective =Effective U=Unknown H=On Hold By Physician	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes							Checked By 1st		
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								Checked By 3rd		
— Chart Codes — Refer to [redacted] last page of the report r a complete list of chart codes) SA=Group Observed - All SI=Observed Individual										

[redacted]

MEDICATI

Admit Date

[redacted]

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

	Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
ANXIETY MEDICATION Lorazepam nitor side effects of Anti-Anxiety ication Q shift. Chart 0= None or Use etter S,A,D,N,V,C,H,B,R: 0= None, edation, A= Ataxia, D=Dizziness, = Drowsiness, N= Nausea, omiting, C= Confusion, H= daché, B= Blurred Vision, R= Skin h y shift	S/E	0	0	0	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Day	[redacted]																												
	S/E	0	0	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Eve	[redacted]																												
	S/E	0	0	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Nig	[redacted]																												
I-DEPRESSANT: Mirtazapine, and italopram. Monitor side effects of Anti- ressant medication Q Shift. Chart 0 = e or use 1st letter S,D,M,B,U,Y,E,A, ,P,W 0= NONE, S=Sedation, Drowsiness, M=Dry Mouth, B= Blurred on, U=Urinary Retention, achycardia, E=Muscle Tremor, gitation, H=Headache, R=Skin Rash, hotosensitivity (Skin), W=Excess ght Gain y shift	SE	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
	*Day	[redacted]																												
	SE	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
	*Eve	[redacted]																												
	SE	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
	*Nigh	[redacted]																												

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
— Follow Up Codes — V=Administered I=Ineffective E=Effective U=Unknown H=On Hold By Physician	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes							Checked By 1st		
								Checked By 2nd		
								Checked By 3rd		
— Chart Codes — Refer to the last page of the report r a complete list of chart codes) BA=Group Observed -All BI=Observed Individual		MEDICAT[redacted]								
		Admit Date								

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
#	1	1	1	1	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
NPI	1,2,3	1,2,5	1,2,3	1,2,3	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
OC	[redacted]	2	2	2	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

avior Monitoring Antianxiety
razepam) - DX anxiety m/b inability to
x. Monitor # Number of [redacted] of
avior observed. Interventions 1-
axation, 2- Adjust room temp/ lighting,
leposition, 4- Toileting, 5- Music/TV,
nack(s)/ Outcomes: 1- Improved, 2-
hanged, 3- Worsened.
ry shift for 14 Days
[redacted]

*Day [redacted]

#	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
NPI	1234	1	1	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
OC	2	2	2	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

*Eve [redacted]

#	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
NPI	1,2,3	123	123	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
OC	1	1	2	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

*Nigh [redacted]

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
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		Admit Date								

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive | POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
#	1	1	1	1	1	1	1	1	1	1	0	0	1	1	1	1	0	0	2	1	1	1	2	2	1				
[redacted]	1,2,3	1,2,5	1,2,3	1,2,3	1	1,2,3	1,2,5	1,2,3	1,2,3	1,2,3	125	246	1,2,3	1,2,3	1,2,6	1,2,6	125	1234	1,2,3	1,2,3	1,2,3	1,2,3	125	124	1,2,6				
OC	[redacted]	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2				
*Day	[redacted]																												
#	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
NPI	1234	1	1	1	0	0	0	1	1	1	123	0	1	1	1	1	1	0	0	1	1	0	1	1					
OC	2	2	2	2	0	0	0	2	2	2	2	0	2	2	2	2	2	0	0	2	2	0	2	2					
*Eve	[redacted]																												
#	0	0	0	1	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
NPI	1,2,3	123	1,2,3	123	NA	NA	1,2,3	123	123	1,2,3	123	NA	NA	123	123	1,2,3	123	123	NA	NA	123	123	123	1,2,3					
OC	1	1	2	0	NA	NA	1	1	2	2	2	NA	NA	2	2	1	2	2	NA	NA	2	2	2	1					
*Nigh	[redacted]																												

uavior Monitoring Antidepressant
(apro). Dx: Depression m/b
)ealization of sadness. Monitor #
nber of episodes of behavior
erved. Interventions: 1- Relaxation, 2-
ust room temp/ lighting, 3- Reposition,
'oileting, 5- Music/TV, 6- Snack(s) /
comes: 1- Improved, 2- unchanged, 3-
'sened.
ry shift

[redacted]

Chart Codes / Follow Up Codes

— Follow Up Codes —
√=Administered
I=Ineffective
E=Effective
U=Unknown
H=On Hold By Physician

— Chart Codes —
[redacted] to the last page of the report
for a complete list of chart codes)
BA=Group Observed - All
BI=Observed Individual
OBP=Group Observed - Partial
1=Absent from facility without meds
2=Drug Refused
3=Absent from facility with meds
4=Vitals [redacted] of Parameters
Administration
5=Hold/See Progress Notes

Init	Name	Signature	Init	Name	Signature		Name	Signature
						Checked By 1st		
						Checked By 2nd		
						Checked By 3rd		

[redacted]

MEDICATI[redacted]

Admit Date [redacted]

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

iavior Monitoring Antidepressant
lication (Remeron). Dx: Depression
poor PO intake less than 50% each
l. Monitor % percentage of each meal
ke. Interventions 1- Encourage to
ease PO intake, 2- Offer alternate
ıu, 3- Allow resident to take his time,
ffer snack as ordered. / Outcomes: 1-
roved, 2- unchanged, 3- Worsened.
r meals
r meals.
[redacted]

Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon
	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29
%	50	50	50	50	100	100	50	50	50	50	100	100	50	50	25	25	100	100	25	25	25	25	100	100	1				
NPI	1,2,6	1,2,5	1,2,6	1,2,3	1,2,6	123	1,2,5	1,2,3	1,2,6	1,2,3	125	245	1,2,6	1,2,6	1,2,6	1,2,6	1,2,6	123	1,2,6	1,2,3	1,2,6	1,2,6	123	123	1,2,6				
OC	[redacted]	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2				
1000	[redacted]																												
%	80	80	80	80	100	100	80	80	80	80	100	100	80	80	80	80	100	100	80	80	80	80	100	100	80				
NPI	1,2,6	1,2,6	1,2,6	1,2,6	1	1	1,2,6	1,2,6	1,2,6	1,2,6	125	125	1,2,6	1,2,6	1,2,6	1,2,3	125	125	1,2,6	1,2,6	1,2,6	1,2,6	125	125	1,2,3				
OC	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2				
1300	[redacted]																												
%	100%	100%	100%	100%	0	0	0	100%	100%	100%	100	0	100%	100%	75%	100%	100%	0	0	100%	100%	0	100%	100%					
NPI	1234	1	1	1	0	0	0	1	1	1	123	0	1	1	1	1	1	0	0	1	1	0	1	1					
OC	2	2	2	2	0	0	0	2	2	2	2	0	2	2	2	2	2	0	0	2	2	0	2	2					
1800	[redacted]																												

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
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		MEDICATI[redacted]								
		Admit Date								

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive | POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

avior Monitoring Antipsychotic
perdal). Dx: psychosis r/t medical
dition m/b sudden mood changes.
itor # Number [redacted] of
avior observed. Interventions 1-
axation, 2- Adjust room temp/ lighting,
eposition, 4- Toileting, 5- Music/TV,
nack(s)/ Outcomes: 1- Improved, 2-
hanged, 3- Worsened.
ry shift
[redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
#	1	1	1	1	1	1	1	1	1	1	0	0	1	1	1	1	0	0	1	1	1	1	0	0	2				
NPI	1,2,3	1,2,5	1,2,3	1,2,3	1	1,2,5	1,2,5	1,2,3	1,2,3	1,2,3	125	23	1,2,3	1,2,3	1,2,6	1,2,6	1234	1234	1,2,3	1,2,3	1,2,3	1,2,3	125	125	1,2,6				
OC	[redacted]	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2	2				
*Day	[redacted]																												
#	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
NPI	1234	1	1	1	0	0	0	1	1	1	123	1	1	1	1	1	1	0	0	1	1	0	1	1					
OC	2	2	2	2	0	0	0	2	2	2	2	0	2	2	2	2	2	0	0	2	2	0	2	2					
*Eve	[redacted]																												
#	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
NPI	1,2,3	123	1,2,3	123	NA	NA	1,2,3	123	123	1,2,3	123	NA	NA	123	123	1,2,3	123	123	NA	NA	123	123	123	1,2,3					
OC	1	1	2	0	NA	NA	1	1	2	2	2	NA	NA	2	2	1	2	2	NA	NA	2	2	2	1					
*Nig	[redacted]																												

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[redacted] for [redacted]

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		1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29
	S/E	1	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
[redacted]	*Day																													
	S/E	0	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Eve																													
	S/E	0	0	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
	*Nigh																													

NOTIC MEDICATION - Monitor side cts of Hypnotic medication Q shift.
rt "0" for None or use [redacted] S,D,
, 0= None, S- Sedation, D-
wsiness, M- Morning Hangover, A-
dia (Drunk walk)
E OF TEMAZEPAM.
ry shift for Use of Temazepam. for 14
s

[redacted]

r crush all crushable meds
vidually.
ry shift
[redacted]

*Day	
*Eve	
*Nigh	

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
--- Follow Up [redacted] --- A=Administered I=Ineffective E=Effective U=Unknown H=On Hold By Physician ------ Chart Codes ------ (Refer to the last page of the report for a complete list of chart codes) BA=Group Observed - All BI=Observed Individual	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes							Checked By 1st Checked By 2nd Checked By 3rd		

[redacted] MEDICATI

[redacted] Admit Date

[redacted] | Pharmacy [redacted] | Physician [redacted]

vance Directive | POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
itor [redacted] of pain Q shift: (Scale [redacted] urrent: 0= No pain, 1-3= Mild Pain, 4-Moderate Pain, 7-10= Severe Pain ry shift	Pain Level	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0				
[redacted]	*Day	[redacted]																											
	Pain Level	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0				
	*Eve	[redacted]																											
[redacted]	Pain Level	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0				
	*Nigh	[redacted]																											
itor pain medication sedation level ry shift - Document: Sleeping Alert Occasionally drowsy, arousable Frequently drowsy, arousable Minimal or no response to stimuli [redacted]	RSLT	1	1	1	1	1	0	0	1	1	1	1	0	0	1	1	1	1	0	0	1	1	1	1	0	0	1	1	1
	*Day	[redacted]																											
	RSLT	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
	*Eve	[redacted]																											
	RSLT	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
	*Nigh	[redacted]																											

[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
ollow Up Codes ----	OBP=Group Observed - Partial							Checked By 1st		
ministered	1=Absent from facility without meds							Checked By 2nd		
ective	2=Drug Refused							Checked By 3rd		
ective	3=Absent from facility with meds									
known	4=Vitals Outside of Parameters									
Hold By Physician	Administration									
	5=Hold/See Progress Notes									
Char [redacted] ----										
to the last page of the report a complete list of chart codes)										
BA=Group Observed -All										
BI=Observed Individual										

[redacted] MEDICATI

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[redacted] for [redacted] 6

	Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon
		1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29
itor S/E of Anti-psychotic Agent. nt "O" for None or use 1st Letters: AP T= Tardive Dyskinesia (Facial, gue movement); C = Cognitive airment (Decreased Mental Status); A cathisia (Inability to Sit Still); P = kinsonism (Tremors, Drooling, dity) ry shift for (Specify medication used) ter Date	CODE	0	0	0	0	0	0	0	0	0	0	2	0	0	0	0	0	0	0	0	0	0	0	0	0	0				
	*Day [redacted]																													
	CODE	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
	*Eve [redacted]																													
	CODE	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0					
	*Nigh [redacted]																													

[redacted]

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								Checked By 2nd		
								Checked By 3rd		
	MEDICATI[redacted]									
	Admit Date [redacted]									

[redacted] Pharmacy [redacted] Physician [redacted]

vance Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

nitor Vital Signs [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
BP	X	X	X	X	X	X	127/ 76	134/ 75	127/ 71	118/ 65	116/ 68	122/ 72	125/ 74	123/ 68	122/ 73	125/ 71	134/ 72	130/ 72	125/ 71	122/ 63	NA	126/ 65	124/ 60	124/ 72	114/ 64				
Temp	X	X	X	X	X	X	98.2	98.2	98.4	98.3	97.6	98	98.2	98.3	98.3	98.1	97.5	97.5	98.2	98.1	98.2	98.2	98.1	97.5	98.2				
Pulse	X	X	X	X	X	X	75	76	72	80	75	74	73	74	76	73	75	76	74	71	NA	73	75	75	82				
Resp	X	X	X	X	X	X	16	16	18	18	18	18	16	16	16	16	18	18	18	16	16	16	18	18	18				
O2 Sats	X	X	X	X	X	X	98	98	98	98	96	98	98	98	97	97	97	97	97	98	NA	97	97	97	98				
*Day	X	[redacted]																											
BP	X	X	X	X	X	126/ 70	122/ 70	122/ 76	115/ 68	127/ 73	126/ 68	117/ 76	126/ 71	134/ 85	126/ 70	132/ 76	120/ 67	NA	126/ 70	115/ 65	110/ 60	117/ 77	122/ 68	126/ 70						
Temp	X	X	X	X	X	98.2	97.6	97.6	97.9	97.9	97.8	97.9	97.9	98.1	97.9	70	97.9	NA	97.6	97.8	97.9	97.8	97.9	97.9						
Pulse	X	X	X	X	X	74	72	82	78	70	72	72	73	80	78	72	72	NA	74	78	72	78	71	74						
Resp	X	[redacted]																											
O2 Sats	X	X	X	X	X	98	98	97	97	97	98	98	97	97	97	97	97	NA	97	98	96	97	97	97						
*Eve	X	X	X	X	[redacted]																								
BP	X	X	X	X	X	128/ 72	132/ 72	134/ 70	116/ 68	128/ 72	126/ 64	126/ 76	130/ 78	130/ 78	128/ 70	134/ 72	122/ 70	126/ 76	130/ 78	126/ 76	114/ 64	118/ 76	120/ 64	135/ 76						
Temp	X	X	X	X	X	97.9	97.8	97.8	97.6	97.6	97.8	97.8	97.9	97.9	97.8	97.8	97.8	97.6	97.5	97.6	97.8	97.8	97.6	97.8						

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
[redacted] ollow Up Codes — ministered fective ective known Hold By Physician Chart Codes — to the last page [redacted] report a complete list of chart codes) A=Group Observed - All I=Observed Individual	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes							Checked By 1st		
								Checked By 2nd		
								Checked By 3rd		
		MEDICATI								
		Admit Date								

[redacted] Pharmacy [redacted] [redacted]

lies: Penicillin

ce Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
...continued from previous page...																													
Pulse	X	X	X	X	X	74	76	74	78	74	74	78	78	78	76	74	74	78	74	72	74	78	72	78					
Resp	X	X	X	X	X	18	18	19	17	18	18	17	20	18	18	18	18	18	18	18	18	18	18	18					
O2 Sats	X	X	X	X	X	95	98	97	96	96	98	97	95	97	97	98	97	97	95	97	96	97	97	98					
*Nigh	X	X	X	X	[redacted]																								
Temp																													
PRN																													
NPI						1									2														
Pain Level						3									4														
PRN					[redacted]																								

...inophen Tablet 325 MG
...ablet by mouth every 4 hours as
...for Temp >100.4°F NTE 3GM
...24 hours from all sources.
[redacted]

...inophen Tablet 325 MG
...ablet by mouth every 6 hours as
...for Pain. Pain level [redacted]
...AP in 24 hours from all sources.
...nt NPI as follows: 1-Relaxation,
...room temp/lighting, 3-
...ion, 4-Toileting, 5-Music/TV, 6-
..., 7-Warm/Cold compress, 8-
[redacted]

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
Follow Up Codes ----	OBP=Group Observed - Partial							Checked By 1st		
Administered	1=Absent from facility without meds							Checked By 2nd		
Ineffective	2=Drug Refused									
Effective	3=Absent from facility with meds									
Unknown	4=Vitals Outside of Parameters									
On Hold By Physician	Administration									
	5=Hold/See Progress Notes									
Chart Codes ----										
(Refer to the last page of the report for a complete list of chart codes)										
OBA=Group Observed - All										
OBI=Observed Individual										

[redacted]

[redacted] Pharmacy [redacted]

Penicillin

ce Directive: POLST; Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
PRN					X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

[redacted]gram Oral Tablet [redacted]MG
[redacted]gram)
[redacted]tablet by mouth every 6 hours as
[redacted]For Anxiety m/b inability to relax
[redacted]ays ; Informed consent obtained
[redacted]m resident.
[redacted]

PRN																													

[redacted]agnesia Oral Suspension 1200
[redacted]G (Magnesium Hydroxide)
[redacted]ml by mouth as needed for No
[redacted]ays. Daily PRN
[redacted]

PRN	X	X	X	X	X															X	X	X	X	X	X	X	X	X	

[redacted]ron HCl Oral Tablet 4 MG
[redacted]n HCl)
[redacted]by mouth every 8 hours as
[redacted]for Nausea and vomiting. for 14
[redacted]

Chart Codes / Follow Up Codes	
[redacted] Follow Up Codes —— [redacted]Administered [redacted]effective [redacted]Effective [redacted]Unknown [redacted]On Hold By Physician [redacted]—— Chart Codes —— (Refer to the last page of the report for a complete list of chart codes) OBA=Group Observed - All OBI=Observed Individual	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters 5=Medication Administration 6=Hold/See Progress Notes

Init	Name	Signature	Init	Name	Signature		Name	Signature
						Checked By 1st		
						Checked By 2nd		

[redacted]

[redacted] Pharmacy [redacted] [redacted]

es: Penicillin

ce Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]

Hours	Mon 1	Tue 2	Wed 3	Thu 4	Fri 5	Sat 6	Sun 7	Mon 8	Tue 9	Wed 10	Thu 11	Fri 12	Sat 13	Sun 14	Mon 15	Tue 16	Wed 17	Thu 18	Fri 19	Sat 20	Sun 21	Mon 22	Tue 23	Wed 24	Thu 25	Fri 26	Sat 27	Sun 28	Mon 29
PRN							X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

etron HCl Oral Tablet 4 MG
setron HCl)
mg by mouth every 8 hours as
for Nausea and vomitting.
Date-

PRN				X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

pam Oral Capsule [redacted]MG
e pam)
mg by mouth every 24 hours as
for Insomnia m/b inability to
of 14 Days
Date-

	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

Equivalents [redacted]e used unless
d
Date-

	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

Full Code, Full Treatment, Long-
tificial Means of Nutrition.
Date-

Chart Codes / Follow Up Codes		Init	Name	Signature	Init	Name	Signature		Name	Signature
---- Follow Up Codes ----	OBP=Group Observed - Partial							Checked By 1st		
/=Administered	1=Absent from facility without meds									
=Ineffective	2=Drug Refused							Checked By 2nd		
E=Effective	3=Absent from facility with meds									
U=Unknown	4=Vitals Outside of Parameters									
H=On Hold By Physician	Administration									
	5=Hold/See Progress Notes									
----- Chart Codes ----										
Refer to the last page of the report										
or a complete list of chart codes)										
OBA=Group Observed - All										
OBI=Observed Individual										

[redacted]

[redacted] Pharmacy [redacted] [redacted]

Penicillin

ce Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for [redacted]6

Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	T
1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29		

Custom Prompt Legend

1. Anticonvulsant Drug level (ACDL)	5. Digoxin Level (Dig)	9. Potassium (K)	13. TSH (TSH)
2. BUN (BUN)	6. Glucose (Glu)	10. PT/INR (PTINR)	14. Urine pH (Ur.pH)
3. CBC (CBC)	7. Hematocrit (Hct)	11. PTT (PTT)	
4. Creatinine (Creat)	8. Hemoglobin (Hgb)	12. Theophylline Level (Theo)	

Staff Administration Le[redacted]

Initial	Long Username	Username	Designation
[redacted]			

Chart Codes / Follow Up Codes

— Follow Up Codes —
=Administered
=Ineffective
=Effective
=Unknown
=On Hold By Physician

— Chart Codes —
Refer to the last page of the report
or a complete list of chart codes)
BA=Group Observed - All
BI=Observed Individual
OBP=Group Observed - Partial
1=Absent from facility without meds
2=Drug Refused
3=Absent from facility with meds
4=Vitals Outside of Parameters
Administration
5=Hold/See Progress Notes

Init	Name	Signature	Init	Name	Signature		Name	Signature
						Checked By 1st		
						Checked By 2nd		

[redacted]

[redacted] Pharmacy [redacted]

ies: Penicillin

ce Directive: POLST: Full Code, Full Treatment, Long-Term Artificial Means of Nutrition.

[redacted] for 6

Hours	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	T
	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29								

[redacted] d from previous page)

Initial	Long Username	Username	Designation
[redacted]			

Chart Codes		
OBA=Group Observed - All	OBI=Observed Individual	OBP=Group Observed - Partial
1=Absent from facility without meds	2=Drug Refused	3=Absent from facility with meds
4=Vitals Outside of Parameters for Administration	5=Hold/See Progress Notes	6=Hospitalized
7=Sleeping	8=Nauseated / Vomiting	9=Other / See Progress Notes
10=Partial Administration	DR=Document Result	

[redacted]

Chart Codes / Follow Up Codes		Init	Name Signature	Init	Name Signature		Name Signature
-- Follow Up Codes -- =Administered =Ineffective =Effective =Unknown =On Hold By Physician -- Chart Codes -- Refer to the last page of the report or a complete list of chart codes) BA=Group Observed - All BI=Observed Individual	OBP=Group Observed - Partial 1=Absent from facility without meds 2=Drug Refused 3=Absent from facility with meds 4=Vitals Outside of Parameters Administration 5=Hold/See Progress Notes					Checked By 1st Checked By 2nd	

[redacted]

[redacted]

Facility Code: —

[redacted]

Location of Administration Report

[redacted]

Time: 16:55:13 PDT

Reside[redacted]

Administration Record: MEDICATION ADMINISTRATION RECORD Sort Residents By: Location Sort Orders By: Schedule Type

No site of administration data found for MEDICATION ADMINISTRATION RECORD

Physical Therapy
Treatment Encounter Note(s)

Provider: [redacted]

[redacted]

Patient:
MRN:

[redacted]

Precautions	Precautions: falls, psych prec.
Pain Assessment	Pain = No pain present, per patient verbal and nonverbal communication
97112	97112: Static and dynamic standing balance (unsupported) with reaching and stepping different directions to increase balance for mobility. No LOB noted.
97116	97116: Gait training without AD on level and uneven surfaces, emphasizing on self pacing for energy conservation and increase safety awareness with directional changes.
97530	97530: transfer training to increase functional task performance.
Response to Tx	Response to Treatment: Activities as tolerated, requires max encouragement to participate with functional activities. Patient later agreeable to gait activities in hallway. NO LOB noted during activities.

[redacted]

Bed Mobility	Bed Mobility = (I); Rolling = (I); Supine --> Sit = (I); Sit --> Supine = (I)
Transfers	Transfers = (I); Sit --> Stand = (I); Bed = DNT; W/C = DNT
Gait	Level Surfaces = (I) (300' x 2); Distance Level Surfaces = 300 feet; Assistive Device = No AD; Uneven Surfaces = (I); Distance Uneven Surfaces = 100 feet; Asstive Device = No AD
Other Areas	Stairs = N/A - Not Applicable at this time
Sitting Balance	Static Sitting = Good; Dynamic Sitting = Good
Standing Balance	Static Standing = Good -; Dynamic Standing = Good -

Original Signature: [redacted]

Date

Cosignature: [redacted]

Date

Missing Consent Signature Missing Consent Signature Missing Consent Signature

[redacted]

[redacted]

[redacted]

Time: 17:05:18 PT

Progress Notes

[redacted]

Resident Name [redacted] Location [redacted] Admission [redacted]

Late Entry
Effective Date: [redacted]
Type: Physician Progress Note

Resident Name [redacted] Location [redacted] Admission Date [redacted]

Note Text: PM&R Progress Note

Mobility/Gait dysfunction: Cont physical therapy for progressive gait training as tolerated with an appropriate assistive device.
Pain Management: Not much pain at this time